ICI & AFROC
Virtual Multidisplinary Tumor Board

Case presentation on Management of Mid-Esophageal SCC with Severe Adult Malnutrition: Challenges on Perioperative Management

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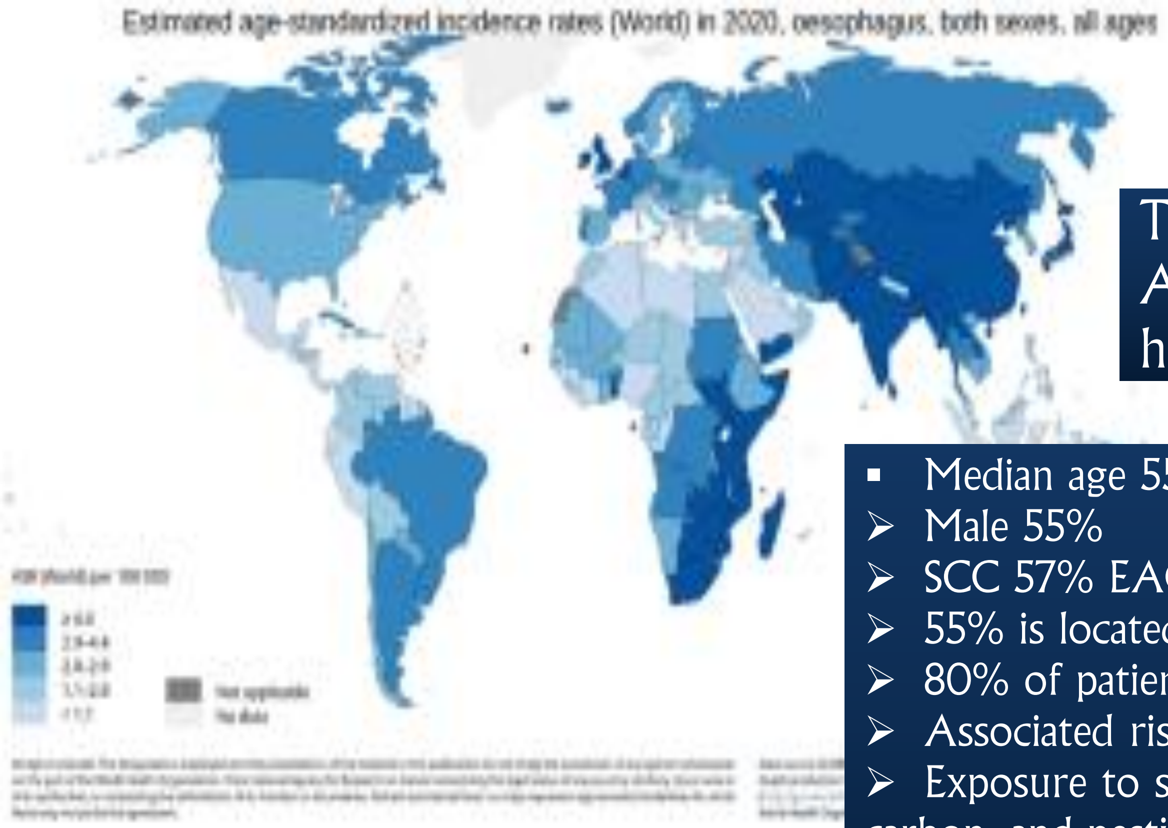
Introduction



Tumor charactrics in Ethiopia (results from A retrospective document review from ten referral hospitals in different regions of Ethiopia. (2012-2017)

- Median age 55years
- Male 55%
- SCC 57% EAC 33%
- 55% is located in mid and lower esophagus
- 80% of patients present as Stage III and IV in their 1st visit
- Associated risk factors smoking and alcohol
- Exposure to silica, asbestos fibre, ionising radiation, polycyclic aromatic hydro-carbon, and pesticides were also observed to increase the risk
- The consumption of hot food and beverages was associated with an increased risk of ESCC
- Common procedure Feeding gastrostomy tube followed by THE

Introduction

Condition	Definition	Key Mechanism	Reversibility
Malnutrition	A state resulting from deficiency or imbalance of energy, protein, and other nutrients causing measurable adverse effects on tissue, function, or clinical outcome.	Inadequate nutrient intake or absorption → negative nitrogen balance → loss of lean body mass.	Usually reversible with adequate nutritional support.
Sarcopenia	Progressive and generalized loss of skeletal muscle mass and strength associated with aging or chronic disease.	Reduced physical activity, hormonal decline, anabolic resistance, mitochondrial dysfunction.	Partially reversible with exercise and nutrition.
Cachexia	A complex metabolic syndrome associated with underlying illness, characterized by muscle wasting with or without fat loss, not fully reversible by nutritional support.	Chronic inflammation, cytokine-driven catabolism, increased energy expenditure, anorexia.	Difficult to reverse, even with nutrition, unless underlying disease is treated.

Case Presentation

Patient HX and PE

- 72 years old male presented with difficulty of swallowing of 5 months duration which is worsened over the past 2 months currently he can take fluid diet
- Also he has significant weight loss and loss of appetite
- He also has history of retrosternal chest pain of the same time duration and intermittent dry cough of 2 weeks duration (he claim it beings after endoscopy)
- No other pertinent history

PE

- ✓ General appearance: CSL(grossly emaciated)
- ✓ HEENT:Prominent zygomatic bones
- ✓ V/S: all with in normal range
- ✓ Weight= 41kg height=166cm BMI=14.9Kg/m2 MAUC=
- ✓ RES: Prominent ribs, clavicles, intercostal spaces, scapular winging
- ✓ Clear with good air entry bilaterally

- ABD
  - no tenderness no palpable mass
  - no sign of fluid collection
  - Scaphoid abdomen
- .GUS no CVAT
- MSS no edema
- CNS COTPP

Investigation
Baseline

- CBC =normal range
- Electrolytes= normal range
- OFT =normal range
- Serum albumin= 3g/dl
- ESR= 80mm/hr
- CRP=41.9mg/dl
- Coagulation profile =normal

ECG = normal
ECO = normal

Investigation
Upper GI endoscopy and biopsy

Endoscopy Report

Report #: 44981

Name	[redacted]	Requesting Institution	
Age	72	Gender	Male
MRN	P-04002-25	Reported on	06-Oct-2025 02:12 PM

Upper GI Endoscopy

Indication : Dysphagia

Premedication : Xylocaine spray, pethidine

Endoscopy Procedure

Hyphopharnx : Normal

Esophagus : Circumferential friable mass noted at 27cms from cental incisor. Scope could not negotaie across the mass

Stomach : Not visualized

Duodenum : Not visualized

Biopsy/Therapy : Multiple biopsies taken from the mass for HPE

Conclusion : ? Mid esophageal Ca

Recommendations: Follow biopsy result

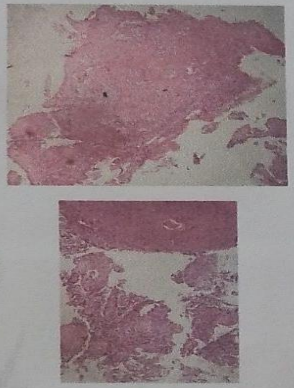
Pathology Report

Report #: 44984

Name	[redacted]	Requesting Institution	
Age	72 | Male	Sample Id	DSH-0050-18
MRN	P-04002-25	Reported on	14-Oct-2025 09:03 AM

BIOPSY LARGE SIZE

Clinical Note	Patient presented with dysphagia. Endoscopy - ?Mid esophageal ca
Gross Finding	0.5cms measuring aggregates of gray white small irregular tissue fragments. All embedded
Microscopy	Serial level sections show dysplastic esophageal mucosa lined tissues with invasive nests, cords, pleomorphic single cell sheets and tongues of malignant squamous cells against a desmoplastic background stroma. Keratin pearls are seen in the middle of tumor nests.
Conclusion	**Mid Esophageal mass; endoscopy biopsy - Invasive Keratinizing Squamous cell carcinoma**
Recommendation	Correlate this with the endoscopy findings.



Imaging

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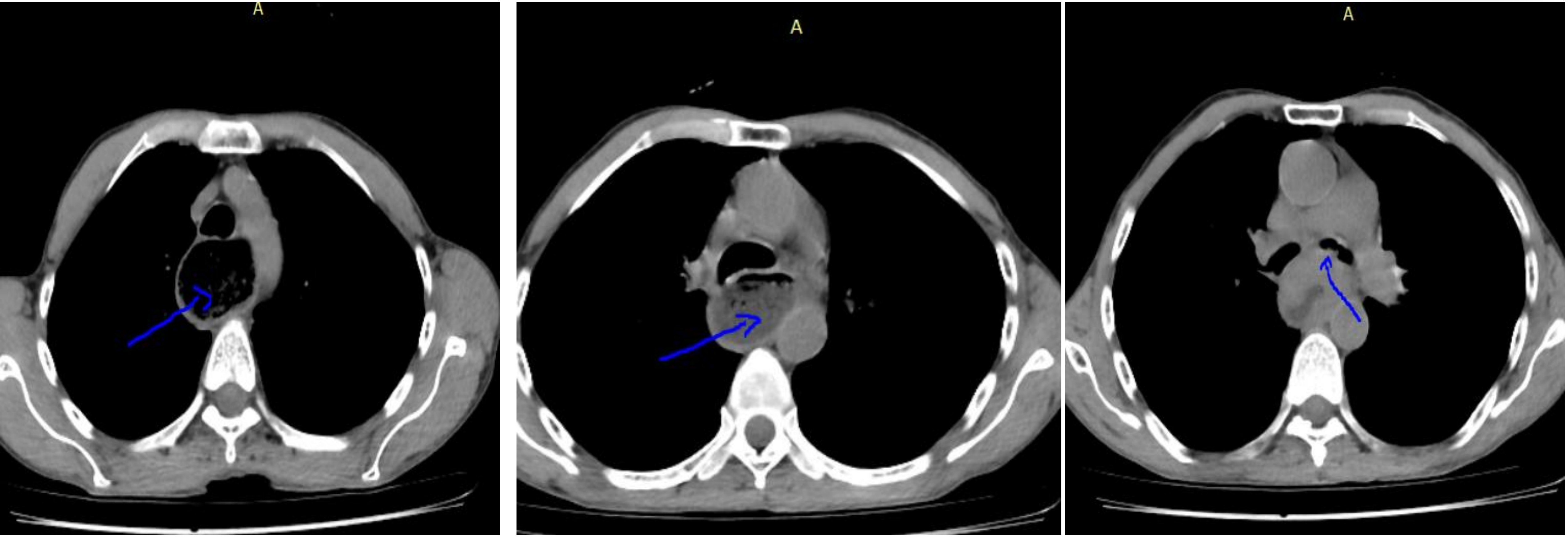


Discussion

Mini Nutritional Assessment
MNA®

Nestlé NutritionInstitute

Last name: First name:

Sex: Age: Weight, kg: Height, cm: Date:

Complete the screen by filling in the boxes with the appropriate numbers. Total the numbers for the final screening score.

Screening

A Has food intake declined over the past 3 months due to loss of appetite, digestive problems, chewing or swallowing difficulties?

0 = severe decrease in food intake
1 = moderate decrease in food intake
2 = no decrease in food intake

B Weight loss during the last 3 months

0 = weight loss greater than 3 kg (6.6 lbs)
1 = does not know
2 = weight loss between 1 and 3 kg (2.2 and 6.6 lbs)
3 = no weight loss

C Mobility

0 = bed or chair bound
1 = able to get out of bed / chair but does not go out
2 = goes out

D Has suffered psychological stress or acute disease in the past 3 months?

0 = yes 2 = no

E Neuropsychological problems

0 = severe dementia or depression
1 = mild dementia
2 = no psychological problems

F1 Body Mass Index (BMI) (weight in kg) / (height in m^2)

0 = BMI less than 19
1 = BMI 19 to less than 21
2 = BMI 21 to less than 23
3 = BMI 23 or greater

IF BMI IS NOT AVAILABLE, REPLACE QUESTION F1 WITH QUESTION F2.
DO NOT ANSWER QUESTION F2 IF QUESTION F1 IS ALREADY COMPLETED.

F2 Calf circumference (CC) in cm

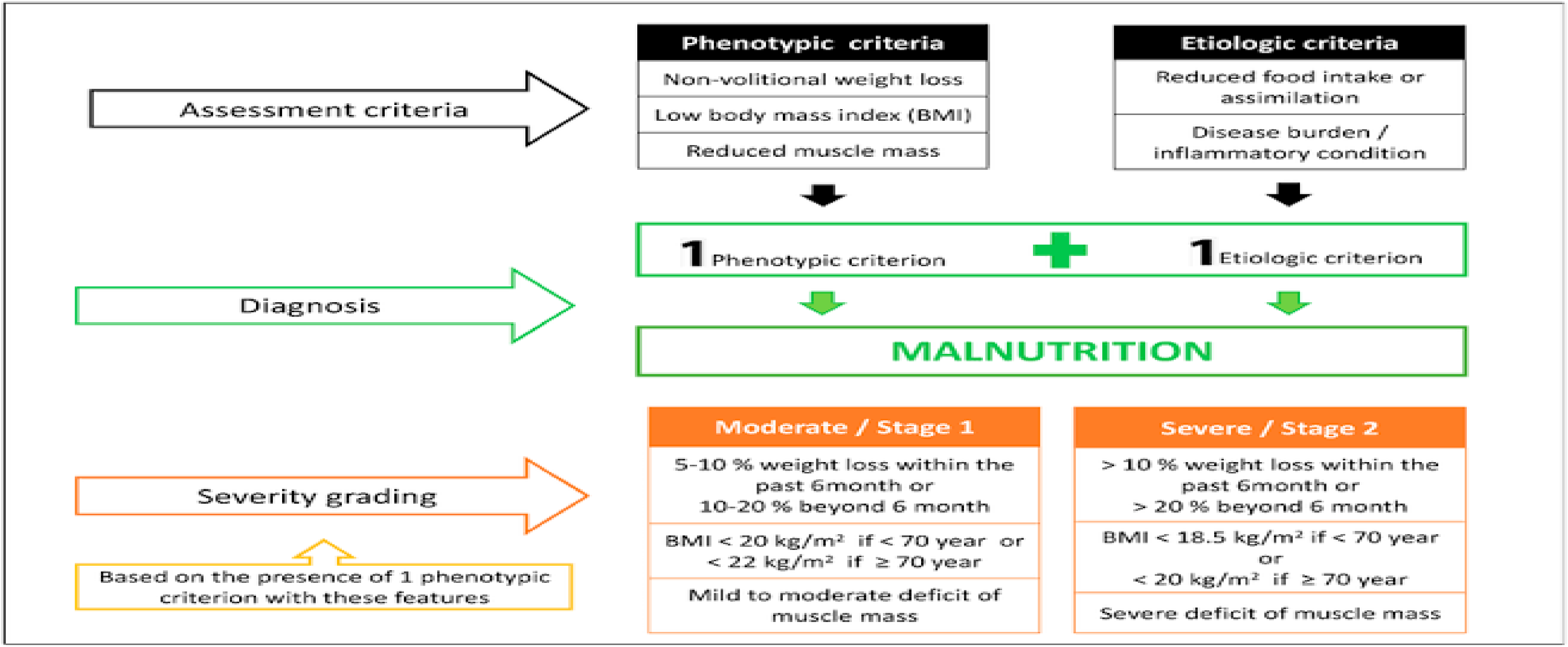
0 = CC less than 31
3 = CC 31 or greater

Screening score (max. 14 points)

12 - 14 points: Normal nutritional status
8 - 11 points: At risk of malnutrition
0 - 7 points: Malnourished

Based MNA our patient is malnourished with score of 4

GLIM Malnutrion Assessment and Severity Grading



Clinical Stage and Assessment

- Stage: cT4bN0M0 (potentially resectable)
- Problem: Severe protein-calorie malnutrition.
- Implications: Poor healing, infection risk, high perioperative mortality.

Plan
Multidisciplinary Approach

- Nutrition: Rehabilitation with enteral/parenteral feeding
- Oncology: Neoadjuvant therapy
- Surgery: Resectability assessment
- Anaesthesia/ICU: Optimize cardiopulmonary and metabolic status

Nutrition Optimization

- Enteral route preferred (gastrostomy,jejunostomy or nasojejunal tube)
- Energy: 30–35 kcal/kg/day; Protein: 1.5–2 g/kg/day
- Duration: 2–4 weeks before therapy
- Monitor albumin, prealbumin, and weight

2.Neoadjuvant Therapy:
Begin only after nutritional optimization

3.Surgical Planning:
Goals: R0 resection, good conduit perfusion, minimize leak risk

Discussion and Questions

MCQs for Webinar Audience

1. Initial management for resectable SCC with severe malnutrition?
   A. Neoadjuvant therapy immediately
   B. Surgery directly
   C. Nutritional rehabilitation (Enteral preferred)
   D. Parenteral chemotherapy
2. Which parameter best reflects short-term nutritional recovery?
   A. Albumin
   B. Prealbumin
   C. BMI
   D. Mid-arm circumference
3. Which of the following is not phenotypic criteria of malnutrition?
   A. Weight loss B. Low BMI C. Reduced muscle mass D. Reduced food intake or Assimilation

Take-Home Messages

- Malnutrition is a modifiable surgical risk.
- Multidisciplinary coordination is key.
- Nutritional optimization = preparation, not delay.